

hovid-Metoclopramide Tablet B.P. 10mg

VIMET02-M2

Description

Round, white uncoated tablet with shallow convex faces, "HD" embossed and scored on the same face.

Composition**Each tablet contains:**

Metoclopramide Hydrochloride B.P. equivalent to Anhydrous Metoclopramide Hydrochloride 10 mg.

Actions and Pharmacology

Metoclopramide Hydrochloride increases gastric motility and accelerates gastric emptying by its dopamine antagonist action. It also has an anti-emetic action that is due partly to its local action on the gastrointestinal tract and partly to a direct action on the central nervous system. Oral absorption is rapid with onset of action about 30 to 60 minutes. It undergoes first-pass hepatic metabolism and renal excretion.

Pharmacokinetics**Renal Impairment**

The clearance of metoclopramide is reduced by up to 70% in patients with severe renal impairment, while the plasma elimination half-life is increased (approximately 10 hours for a creatinine clearance of 10-50 mL/minute and 15 hours for a creatinine clearance <10 mL/minute).

Hepatic Impairment

In patients with cirrhosis of the liver, accumulation of metoclopramide has been observed, associated with a 50% reduction in plasma clearance.

Indications

- To control nausea and vomiting associated with intolerance to essential drugs possessing emetic properties (such as cytotoxic agents); uraemia, radiation sickness, malignant disease, post-operative vomiting.
- Management of gastric retention after gastric surgery.
- Pylorospasm.
- As adjunct to X-ray examination of stomach and duodenum to assist in intestinal intubation.

Contraindications

- Avoid in patients hypersensitive to procaine and procainamide.
- Not recommended in patients with epilepsy; gastrointestinal haemorrhage, intestinal obstruction, or perforation; and phaeochromocytoma.

Precautions

- Extrapyramidal effects of metoclopramide are more likely to occur in children and elderly patients following high doses over a long period of time.
- Concurrent use with other medications causing extrapyramidal reaction, especially the phenothiazines, may increase the frequency and severity of extrapyramidal effects.
- Caution in patients with liver or chronic renal failure.
- This medicine may cause drowsiness. Caution if taking alcohol or other CNS depressants.
- Safety for use in pregnancy and lactation has not been established.

Pregnancy and Lactation**Pregnancy**

A large amount of data on pregnant women (more than 1000 exposed outcomes) indicates no malformative toxicity nor fetotoxicity. Metoclopramide can be used during pregnancy if clinically needed. Due to pharmacological properties (as other neuroleptics), in case of metoclopramide administration at the end of pregnancy, extrapyramidal syndrome in newborn cannot be excluded.

Metoclopramide should be avoided at the end of pregnancy. If metoclopramide is used, neonatal monitoring should be undertaken.

Breastfeeding

Metoclopramide is excreted in breast milk at low level. Adverse reactions in the breast-fed baby cannot be excluded. Therefore metoclopramide is not recommended during breastfeeding. Discontinuation of metoclopramide in breastfeeding women should be considered.

Main Side/Adverse Effects

Drowsiness, restlessness, fatigue, bowel upsets, gynaecomastia, dizziness, headache, galactorrhoea, anxiety and skin rash.

Drug Interactions

Caution in patients receiving the following drug therapy :

Acetaminophen, levodopa, tetracycline, anticholinergics, narcotic analgesics, bromocriptine, digoxin.

Overdose and Treatment**Symptoms**

Extrapyramidal disorders, drowsiness, decreased level of consciousness, confusion, hallucination, and cardio-respiratory arrest may occur.

Management

In case of extrapyramidal symptoms related or not to overdose, the treatment is only symptomatic (benzodiazepines in children and/or anticholinergic anti-parkinsonian medicinal products in adults). A symptomatic treatment and a continuous monitoring of the cardiovascular and respiratory functions should be carried out according to clinical status.

Dosage and Administration

Adults : Oral, the equivalent of 10mg of anhydrous metoclopramide hydrochloride three times daily.

Total daily dose of metoclopramide, especially for children and young adults, should not normally exceed 0.5 mg/kg body weight.

Warning

Avoid doses exceeding 0.5 mg/kg/day.

Extrapyramidal effects, especially dystonic reactions of metoclopramide are more likely to occur in children shortly after initiation of therapy, and usually with doses higher than 0.5 mg per kg of body weight per day.

Storage : Store below 25°C. Protect from light and moisture.

Presentation/Packing : Tablet 10 mg x 1000's, 500's, 100's,

Product Registration Holder : HOVID Bhd., 121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

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Revision date : June 2015